

BlueCross BlueShield Appeals and Grievances Handbook

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Introduction

This handbook explains the procedures and your rights when you disagree with a BlueCross BlueShield (BCBS) decision about your health coverage. The document covers denials and adverse determinations, the internal appeal process, independent external review, and the grievance system. Understanding these pathways is essential to protecting your benefits and ensuring you receive the care to which you are entitled. Throughout, we highlight specific plan values that affect how claims are processed, enabling you to use the correct figures in any dispute.

All timelines and rules described here are subject to the terms of your Benefit Agreement and the Policy Document w14_bluecross_blueshield_detailed_policy_018, especially the section **Prior Authorization and Utilization Review**.

Plan Reference Card

The following parameters define your coverage. Each value appears exactly as it applies to your plan:

- in_network_deductible: \$1,200
- out_of_pocket_max: \$5,800
- er_copay: \$200
- mri_prior_auth: Not required for in-network
- mental_health_visits: 30/year, \$35 copay
- pre_existing_waiting: None
- out_of_network_coverage: 70% after \$2,500 deductible
- prescription_tiers: 4 tiers
- xref_policy_doc: w14_bluecross_blueshield_detailed_policy_018
- xref_policy_appeal_section: Prior Authorization and Utilization Review

Please refer to these values whenever you review claim denials or prepare an appeal.

1. Understanding Denials and Adverse Determinations

1.1 What Is a Denial?

A denial occurs when BCBS determines that a claim for payment or a request for a health care service cannot be approved as submitted. Reasons range from lack of medical necessity and benefit exclusions

to exceeding plan limitations, out-of-network care, or failure to obtain required prior authorization. You and your provider will receive an Explanation of Benefits (EOB) or a formal decision letter detailing the specific reason and your appeal rights.

1.2 Adverse Determination vs. Claim Denial

An adverse determination is a formal term for a decision to deny, reduce, or terminate a service based on medical necessity, appropriateness, health care setting, or level of care. Not every claim denial is an adverse determination; for instance, a denial because your in-network deductible of \$1,200 has been fully met but the cost-share is still applicable is a benefit administration action, not a medical judgment. However, both types of denials can be appealed through the internal process in Section 2.

1.3 Common Grounds for Denial

Understanding why a denial happened is the first step toward overturning it:

- **Medical Necessity:** BCBS may determine that a requested service, supply, or drug is not medically necessary. For example, a physician may order an MRI scan that is not supported by conservative treatment trials. While MRI prior authorization is: Not required for in-network facilities, the scan may still be denied if clinical documentation does not justify its medical necessity.
- **Prior Authorization Requirements:** Many services require prior authorization. If the provider fails to obtain it before delivery (unless it is an emergency), the claim will be denied. However, for in-network MRI, remember that MRI prior authorization is: Not required for in-network.
- **Benefit Limits:** Your plan has numerical limits. For example, mental_health_visits: 30/year, \$35 copay. Once you exceed 30 visits in a calendar year, additional visits will be denied, even if they are considered medically necessary.
- **Out-of-Network Care:** When you use a non-participating provider, your plan covers services at out_of_network_coverage: 70% after \$2,500 deductible. The plan pays based on the usual, customary, and reasonable (UCR) rate, and you are responsible for the balance, including any amount above UCR. Denials may arise if services are deemed not covered when out-of-network.
- **Experimental/Investigational Services:** Treatments not yet proven effective through clinical evidence are typically excluded. These denials are often subject to external review.
- **Coding and Billing Errors:** Incorrect ICD-10 codes, CPT codes, or modifiers can result in denials. These are often corrected by the provider, but if the provider does not cooperate, you may need to file an appeal.
- **Pre-Existing Condition Exclusions:** The Affordable Care Act prohibits denials based on pre-existing conditions. In accordance with federal law, your plan has pre_existing_waiting: None. No claim may be denied for that reason; if you receive a denial that mentions a pre-existing condition, you should immediately file a grievance.

1.4 Reading Your Explanation of Benefits (EOB)

Your EOB is a statement of how a claim was processed; it is not a bill. It lists:

- Billed charges
- Allowed amount
- Amount paid by BCBS
- Your cost-sharing: deductible, copay, coinsurance

- Denial reason and code, if applicable

For example, after an emergency room visit, you will see the plan's er_copay: \$200 applied. If you have not yet met your in_network_deductible: \$1,200, the deductible balance will appear. Understanding these numbers helps you verify that the out_of_pocket_max: \$5,800 is being tracked accurately -- once you reach that maximum, BCBS pays 100% of covered in-network services for the rest of the plan year.

1.5 Prior Authorization and Utilization Review

The utilization review (UR) process determines whether a proposed service meets coverage criteria. Your provider initiates a request by submitting clinical records. BCBS reviews standard requests within 15 calendar days and expedited (urgent) requests within 3 business days. The detailed rules are in your xref_policy_doc: w14_bluecross_blueshield_detailed_policy_018, under the section xref_policy_appeal_section: Prior Authorization and Utilization Review. If the request is denied, you receive an adverse determination letter with appeal instructions.

1.6 Types of Adverse Determinations

- **Pre-Service (Prospective):** Denial of a service not yet provided. Either standard or expedited.
- **Concurrent Review:** Denial of an ongoing stay or course of treatment.
- **Post-Service (Retrospective):** Denial after the service has been delivered.

The type affects the timeframe for your internal appeal.

1.7 How Denials Relate to Your Financial Responsibility

Understanding your cost-sharing values is critical when evaluating a denial. The table below shows how a denied claim may be reprocessed if overturned.

Event	Amount / Rule
In-network deductible (individual)	\$1,200
Out-of-pocket maximum (in-network)	\$5,800
ER copay	\$200
In-network coinsurance after deductible	Usually 20% until out-of-pocket max
Mental health office visit copay (in-network)	\$35 (up to 30 visits)
Out-of-network coverage rate	70% after \$2,500 deductible
Prescription drug tiers	4 tiers, variable copays/coinsurance

For example, if a claim for a covered in-network service is denied but later reversed, your cost-sharing will be recalculated. If you have already satisfied your in_network_deductible: \$1,200 and are in the coinsurance phase, you pay a percentage until you hit the out_of_pocket_max: \$5,800. Once that maximum is reached, BCBS pays 100% for the remainder of the year.

1.8 Real-World Denial Scenarios

- **Scenario 1:** A member requests an in-network MRI for chronic knee pain without prior physical therapy. The plan's medical necessity guidelines are not met, so the claim is denied. Because mri_prior_auth: Not required for in-network, the denial is based on medical policy, not lack of authorization.

- **Scenario 2:** A member seeks her 31st mental health therapy visit. Because mental_health_visits: 30/year, \$35 copay, the 31st visit is denied as exceeding the benefit limit. An appeal may be filed if the visits were medically necessary to prevent a crisis, invoking mental health parity protections.

- **Scenario 3:** A member receives out-of-network surgery. The plan applies out_of_network_coverage: 70% after \$2,500 deductible. The denial of part of the billed charges (the amount above UCR) is consistent with the plan, but the member may appeal if a network deficiency existed.

2. Internal Appeal Process

2.1 Your Right to an Internal Appeal

You may request a full and fair review of any adverse determination or claim denial. The internal appeal is handled by BCBS at no cost. You can submit additional evidence and arguments, and a decision will be made by individuals not involved in the original denial.

2.2 Categories of Internal Appeals

1. **Standard Pre-Service Appeal:** For denial of a service not yet received, decided within 15 calendar days (10 days for urgent but not life-threatening).

2. **Standard Post-Service Appeal:** For denial of a claim for services already provided, decided within 30 calendar days.

3. **Expedited Appeal:** For situations where a delay would seriously jeopardize your life, health, or ability to regain maximum function. Decided within 72 hours.

2.3 Filing Deadlines and Requirements

You must file your internal appeal within **180 calendar days** of the denial notice. Appeals not submitted by this deadline may be dismissed unless you can demonstrate good cause for the delay.

Required elements:

- Member name, date of birth, and ID number
- Denial letter reference number and date
- A statement explaining why the denial is incorrect
- Supporting documents (e.g., medical records, physician letters, clinical guidelines)
- Signed authorization if a representative is filing on your behalf

Submit the appeal to: **BlueCross BlueShield Appeals Department** P.O. Box 11045 Health Plan City, ST 12345 Fax: (800) 555-0199 Online: www.bcbs.com/memberappeals

2.4 Two-Level Internal Appeal Structure

If your first-level appeal is denied, you have the right to a second-level internal review.

- **Level 1 - Reconsideration:** Reviewed by a panel of clinical and non-clinical staff not involved in the original decision. For medical necessity denials, a board-certified physician in the relevant specialty will conduct the review.

- **Level 2 - Internal Review Committee:** If Level 1 is unfavorable, you may request Level 2 within **60 calendar days** of the Level 1 decision. This review is conducted by a committee that includes senior medical directors. You may request a telephone hearing to present your case.

The decision timelines for Level 2 are identical to those for Level 1 (15 days for pre-service, 30 days for post-service).

Table 2-A: Internal Appeal Process Summary

Step	Action	Deadline to File	Decision Issued By
Level 1 - Standard	Written appeal with supporting documents	180 days from denial letter	15 days (pre-service) / 30 days (post-service)
Level 1 - Expedited	Urgent request by phone or online	180 days; urgent criteria must be met	72 hours
Level 2 - Internal Committee	Request second review after Level 1 denial	60 days from Level 1 denial	Same as Level 1

2.5 Expedited Appeal Special Rules

To request an expedited appeal, call (800) 555-0198 and state that your situation is urgent. Your physician may also submit a written statement supporting the urgency. We will render a decision within 72 hours, even if all records are not available. If we do not meet that deadline, you may proceed directly to external review.

2.6 What to Expect During the Appeal

- **Acknowledgment:** Within 5 business days, you will receive confirmation that your appeal was received.

- **Investigation:** We gather all relevant records and may consult independent experts.

- **Decision:** A written decision will explain the rationale, cite plan provisions and clinical evidence, and inform you of further appeal rights including external review.

2.7 Appeal Examples Using Your Plan Specifics

- **Prescription Drug Tier Change:** Your pharmacy benefit has prescription_tiers: 4 tiers. If your doctor prescribes a Tier 3 non-formulary drug and you believe Tier 2 alternatives are not appropriate, you can appeal the tier assignment. Provide lab results, prior medication failures, and a letter of medical necessity.

- **Out-of-Network Reclassification:** Suppose you received emergency care at an out-of-network hospital and BCBS processed the claim as out-of-network (out_of_network_coverage: 70% after \$2,500 deductible). If you believe the situation warranted in-network rates because no in-network hospital was accessible, you may appeal